

[REDACTED], MD

[REDACTED], Inc.

[REDACTED] Street, #2 [REDACTED]

San [REDACTED], CA 9[REDACTED]

hello@[REDACTED].com

[REDACTED] - [REDACTED] - [REDACTED]

FROM THE OFFICE OF [REDACTED] MD MPH

April 20, 2026

Patient: Nicolas Hernandez

Date of Birth: [REDACTED]

Re: Medical Necessity Statement for Expedited Review

I am writing on behalf of our patient, who is currently under care for a medically supervised benzodiazepine taper, which is being conducted due to physiological dependence and a history that includes significant withdrawal symptoms and elevated risk during medication transitions.

Interruptions in care, including delays in access to prescribed medications or discontinuation of treatment due to financial barriers, carry a clinically significant risk. These risks include acute withdrawal symptoms, destabilization of the central nervous system, and the potential for life-threatening complications such as seizures. Additionally, abrupt disruption may negatively impact mental health stability and the patient's ability to maintain daily functioning.

Given the nature of benzodiazepine dependence and the need for a carefully managed, gradual taper, timely and uninterrupted access to care is medically necessary. Delays associated with standard processing timelines may pose a risk to the patient's physical and mental health, as well as their ability to maintain and regain functional stability.

Based on the above, it is my clinical opinion that this situation meets criteria for expedited review. Please feel free to contact me if additional information is needed.

Sincerely,

[REDACTED]

[REDACTED], PMHNP-BC

Collaborating Physician: [REDACTED], MD MPH, Family Medicine

[REDACTED], MD

[REDACTED], Inc.

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[REDACTED] - [REDACTED] - [REDACTED]

FROM THE OFFICE OF [REDACTED] MD MPH

Visit Summary: March 27, 2026

Overall

- Ongoing stress related to insurance reimbursement and legal action against LA Care; DHCS Conlan Department involved
- Filed small claims due to lack of response for expedited hearing
- Financial constraints impacting ability to continue [REDACTED] program if reimbursement not approved
- Remains highly engaged in care and motivated to continue taper if financially able

Medical

- No withdrawal symptoms or instability
- No new medical concerns discussed

Taper

- Previous dose, diazepam 20 mg/day, 01/18/2026
- Current dose, diazepam 17.5 mg/day, started 03/03/2026
- Planned next dose, diazepam 15mg/day, planned start 04/06/2026
- Anticipating a dose hold of approximately 4 weeks prior to next reduction to diazepam 10mg/day, anticipated on or after 04/17/2026

Refill

- Prescription sent to CVS 03/30/2026 for diazepam 17.5 mg/day, 7 days
- Subsequent Rx send to CVS for diazepam 15 mg/day, 7 days

Follow-Up Video Visit: Friday, May 1, 12:30p (PT)

[REDACTED], MD

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[REDACTED]-[REDACTED]-[REDACTED]

FROM THE OFFICE OF [REDACTED] MD MPH

Visit Summary: Feb 25, 2026

Overall

- More stable compared to prior week
- Pharmacy filled last prescription two days early creating buffer
- No current pharmacy conflicts
- DHCS reimbursement form 4521 received and under review
- If reimbursement approved plans to extend taper to approximately 12 months
- Feels ready to move forward with next reduction
- Prefers smaller reductions to avoid rebound or destabilization

Medical

- No acute withdrawal symptoms reported
- Continues healing from prior pharmacy-related stress episode
- No instability noted at current dose

Taper

- Current dose, diazepam 20 mg/day, date started 01/18/2026
- Planned next dose, diazepam 17.5 mg/day, planned start date 03/03/2026

Refill

- Prescription sent to CVS 03/02/26
- Update 03/09/26: Subsequent Rx sent to CVS for diazepam 17.5 mg/day, 7 days

- Follow-Up Video Visit: Friday, March 27, 11:30a (PT)